

Massive Rotator Cuff Tears

From repair to replace

10-11 September 2027

Acibadem Altunizade Hospital

Istanbul, Turkey

SCIENTIFIC PROGRAMME

1.5 days

Scientific programme
Friday and Saturday morning

6

Scientific sessions
plus the grand finale debate

4

Relive surgical videos
on day one

TBC

CME credits
details to follow

SEP

10

FRI

Day one

08:30 to 18:30 • 4 sessions • 21 talks

08:30 to 10:30

120 MIN

Anterosuperior tears • Repairable massive tears

FORMAT Expert presentations, a “Relive” surgical video and case-based discussion

- 1 Open versus arthroscopic repair: selecting the appropriate approach
- 2 “Relive” surgical video: How I repair it
- 3 Augmentation techniques
- 4 Management of the pseudoparetic shoulder
- 5 Case-based discussion: two clinical cases

10:30 to 11:00

Coffee break • 30 min

11:00 to 13:00

120 MIN

Anterosuperior tears • Irreparable tears

FORMAT An expert debate, three “Relive” surgical videos and case-based discussion

- 1 Expert debate: Which tendon transfer?
- 2 Tendon transfer selection: evidence-based indications and outcomes
- 3 “Relive” surgical video: Pectoralis minor transfer
- 4 “Relive” surgical video: Pectoralis major transfer
- 5 “Relive” surgical video: Latissimus dorsi transfer
- 6 Case-based discussion: two clinical cases

13:00 to 14:00

Lunch break • 60 min

14:00 to 16:00

120 MIN

Posterosuperior tears · Repairable massive tears**FORMAT** Expert presentations and case-based discussion

- 1 Evidence-based treatment algorithm
- 2 Biological augmentation techniques: footprint preservation, growth factors and stem cells
- 3 Biceps augmentation: techniques and outcomes
- 4 Patch augmentation: technique and outcomes
- 5 Case-based discussion: two clinical cases

16:00 to 16:30

Coffee break · 30 min

16:30 to 18:30

120 MIN

Posterosuperior tears · Irreparable tears: techniques and outcomes**FORMAT** Expert presentations and case-based discussion

- 1 Functional repair
- 2 Superior capsular reconstruction using the long head of the biceps
- 3 Superior capsular reconstruction using fascia lata autograft
- 4 Tendon transfers: latissimus dorsi versus lower trapezius
- 5 Case-based discussion: two clinical cases

SEP

11

SAT

Day two

08:30 to 13:00 · 2 sessions · 11 talks

08:30 to 10:30
120 MIN

Session I · When arthroscopy is no longer enough

DISCUSSION Five presentations, followed by 30 minutes of open discussion

- 1 Indications and patient selection: massive tears, retears and pseudoparalysis
- 2 Biomechanics: the influence of the residual rotator cuff
- 3 From preoperative planning to virtual reality
- 4 Indications for tendon transfer
- 5 Complications

10:30 to 11:00

Coffee break · 30 min

11:00 to 13:00
120 MIN

Session II · Complex clinical cases

FORMAT An interactive discussion of four complex clinical cases, with 30 minutes for each case. Case details to be announced.

Time TBC
TO BE CONFIRMED

Grand finale debate

CHAIRS Emmanouil Brilakis and Maristella Saccomanno · What is the best treatment for the 60-year-old active patient with an irreparable posterosuperior rotator cuff tear? Proposed panel, participation to be confirmed.

- 1 Superior capsular reconstructionTeruhisa Mihata
- 2 Lower trapezius transferBassem Elhassan
- 3 Functional (partial) repairEmilio Calvo
- 4 Reverse shoulder arthroplastyFelix Savoie III

Time TBC
TO BE CONFIRMED

Keynote lectures

SCHEDULING Dates and times to be confirmed

- 1 The Future of Rotator Cuff ReconstructionSpeaker to be confirmed
- 2 Massive Rotator Cuff Tears: Repair, Reconstruct, Transfer or Replace?Emilio Calvo, proposed

FACULTY

CHAIRS

Emmanouil Brilakis ESSKA SHOULDER SECTION CHAIR	Maristella Francesca Saccomanno SCIENTIFIC CO-CHAIR	Alfonso Maria Romano SCIENTIFIC CHAIR	Bariş Kocaoğlu LOCAL CHAIR
Nezih Ziroglu LOCAL CHAIR	Kerem Bilsel LOCAL CHAIR		

PROPOSED SPEAKERS

Teruhisa Mihata JAPAN	Bassem Elhassan US	Emilio Calvo SPAIN	Felix Savoie III US
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Preliminary programme, subject to change. Session moderators will be announced, and faculty participation, lecture scheduling and programme details remain subject to confirmation.